

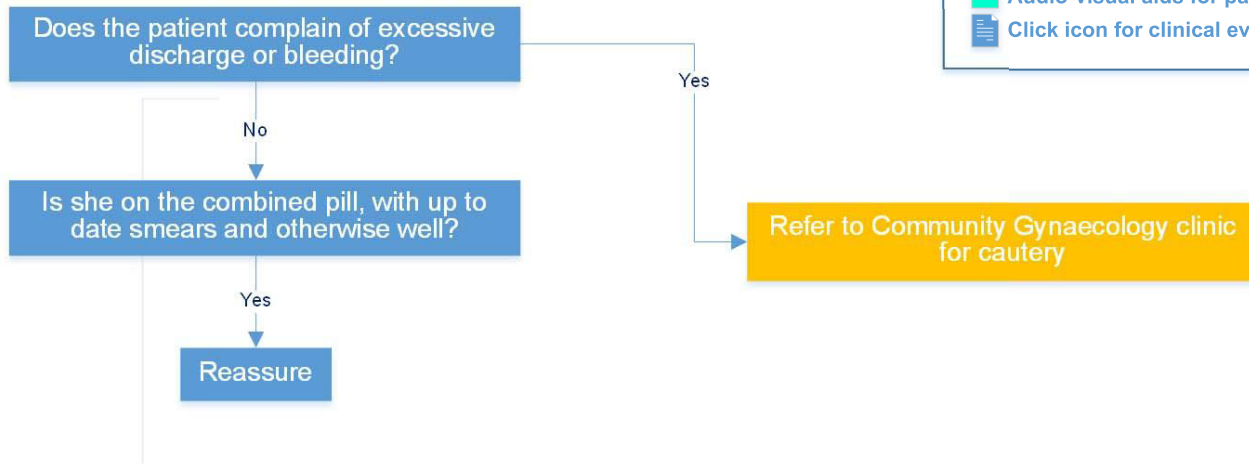
Benign Cervical Problems

Version 1.0: November 2018
Review Date: November 2019

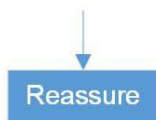
Key

- “Must do” actions for GPs (Triaged by Enfield Referral Service)
- Recommendations for Primary Care
- Red flag / urgent referral
- Routine referral
- Public health intervention
- Audio-visual aids for patients and GP
- Click icon for clinical evidence

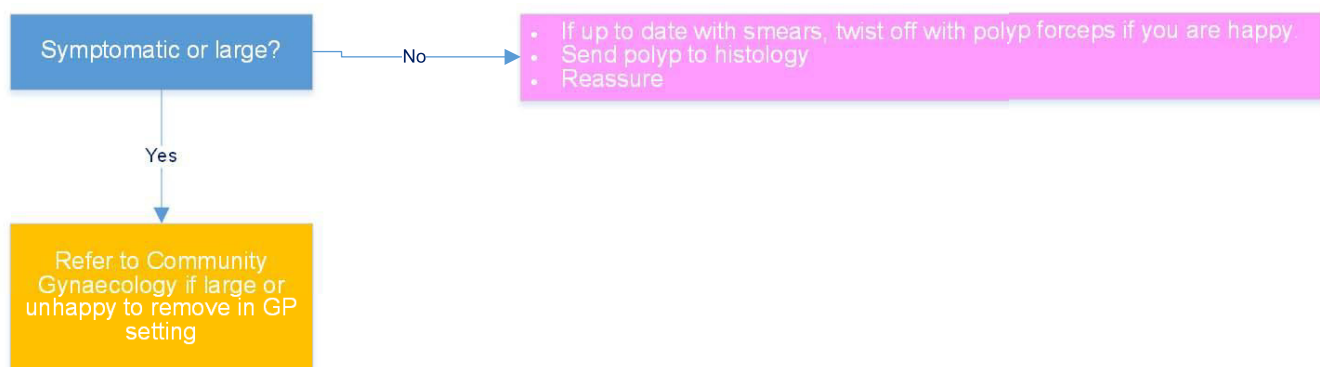
Ectropion



Nabothian Follicles



Endocervical polyps



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Key points:

- Most young women (particularly if they are taking the combined oral contraceptive pill) have an ectropion also known as a cervical erosion.
- Nabothian follicles are always benign.
- Incidental cervical polyps can safely be removed in primary care but (to be safe) always send histology and arrange a check ultrasound scan.

Notes:

1. Most young women, particularly if they are taking the combined oral contraceptive pill, have a cervical erosion (ectropion) which may be observed during routine pelvic examination. If there are no symptoms, there is no reason to offer treatment. Women experiencing a heavy discharge or postcoital bleeding may be referred to the Colposcopy Clinic for cautery.
2. Nabothian follicles (or cysts) are mucus-filled lumps on the surface of the cervix. They are typically small, smooth and rounded. They may be multiple but are always benign.
3. Endocervical polyps are most usually found in the fourth to sixth decade of life. They are cherry red, single or multiple and appear as a pedunculated lesion on a stalk of varying length. Large, symptomatic endocervical polyps should be referred for further evaluation. Small, asymptomatic polyps protruding from the cervical os may safely be removed, but check histology.